

Women's Specialists of Clear Lake

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Hypertension in Pregnancy

Understanding preeclampsia, gestational hypertension, and chronic hypertension

TYPES OF HYPERTENSION IN PREGNANCY

| Condition       | Definition                                                 |
|-----------------|------------------------------------------------------------|
| Chronic HTN     | BP ≥140/90 before 20 weeks                                 |
| Gestational HTN | BP ≥140/90 after 20 weeks, no proteinuria                  |
| Preeclampsia    | BP ≥140/90 + protein in urine or organ damage after 20 wks |
| Eclampsia       | Preeclampsia + seizures                                    |
| HELLP Syndrome  | Hemolysis, elevated liver enzymes, low platelets           |

RISK FACTORS FOR PREECLAMPSIA

- First pregnancy or new partner
- Obesity (BMI ≥30)
- Age <18 or >40
- Multiple pregnancy (twins/triplets)
- Chronic hypertension or kidney disease
- Diabetes (type 1, 2, or GDM)
- Prior preeclampsia
- Autoimmune conditions (lupus, antiphospholipid syndrome)

SYMPTOMS OF PREECLAMPSIA

- Severe headache not relieved by Tylenol
- Vision changes — blurring, flashing lights, spots
- Sudden swelling of face, hands, or feet
- Severe upper abdominal or shoulder pain
- Nausea/vomiting in 2nd or 3rd trimester
- Shortness of breath (fluid in lungs)
- May be asymptomatic — detected at appointments

PREVENTION & MONITORING

- Low-dose aspirin (81 mg/day) starting at 12–16 weeks for high-risk women **RX**
- Calcium supplementation if low dietary intake
- Regular BP checks at every prenatal visit
- Home BP cuff recommended for high-risk patients
- Urine protein screening each trimester
- Weight and swelling assessment at each visit

TREATMENT OPTIONS

- **Labetalol** — BP control in acute/chronic settings **RX**
- **Nifedipine** — calcium channel blocker **RX**
- **Hydralazine** — IV for hypertensive urgency **RX**
- **Magnesium sulfate** — seizure prevention in severe preeclampsia **RX**
- **X AVOID:** ACE inhibitors, ARBs — harmful to fetus
- Delivery is the only cure for preeclampsia

DELIVERY TIMING

- **Gestational HTN without severe features:** 37 weeks
- **Preeclampsia without severe features:** 37 weeks
- **Preeclampsia with severe features:** 34 weeks or sooner
- **HELLP syndrome or eclampsia:** Immediate delivery
- Postpartum monitoring critical — BP can worsen days after delivery

**EMERGENCY SIGNS — GO TO L&D OR CALL 911 IMMEDIATELY**

- BP ≥160/110 on home cuff — take a second reading
- Severe headache unrelieved by Tylenol
- Vision changes, flashing lights, or temporary blindness
- Sudden severe upper abdominal pain
- Difficulty breathing or chest tightness
- Seizure or loss of consciousness
- Decreased fetal movement
- Sudden swelling of face or hands

**With Close Monitoring, Most Women Have Healthy Outcomes**

Hypertensive disorders of pregnancy are among the most monitored and managed conditions in obstetrics. When caught early and treated appropriately, most women — and their babies — do very well. Keep all your prenatal appointments, monitor your blood pressure at home if instructed, and never hesitate to call us with concerns.